

MEDILAB

Customer Service, Complaints, Refunds & Escalation Policy

Knowledge Base Category: Customer Service

Document Type	Customer-facing process / policy guidance
Intended Use	RAG knowledge retrieval and website reference
Version	1.0 - Assessment Dataset
Clinical Boundary	Operational information only; not clinical advice

A structured policy for handling service questions, complaints, billing concerns, refunds, and escalation while preserving clinical boundaries.

Quick Reference

Use this document for	Do not use this document for
Stable customer-service process, policy, preparation, FAQ, and operational guidance.	Live prices, slot availability, booking status, patient-specific clinical interpretation, diagnosis, or medication advice.

1. Service Principle

MediLab customer service should provide clear, respectful, evidence-based operational support. Staff should confirm the relevant booking, test, policy, or knowledge source before making a factual statement about the customer's case.

If information is uncertain or the system does not contain the answer, staff should say so and escalate rather than guess.

2. Types of Customer Requests

Common requests include test and package information, branch or home-visit process questions, preparation instructions, booking changes, booking status, result-access issues, service complaints, billing questions, and policy clarification.

Clinical interpretation, diagnosis, medication advice, emergency triage, and symptom-based test recommendations are outside the customer-service scope.

3. Complaint Intake

Record the customer's contact details, relevant booking or order reference, date and location of service, a concise description of what happened, and the requested resolution. Avoid collecting unrelated sensitive information.

Acknowledge the complaint and explain the next step. Do not promise a specific outcome before the facts and relevant system records are reviewed.

4. Priority Complaints

Escalate promptly when a complaint involves suspected patient misidentification, privacy disclosure, repeated failed collection, possible specimen mix-up, aggressive or unsafe behavior, a serious billing discrepancy, or a system transaction that may have affected the wrong booking.

Customer-service escalation is operational. Medical emergencies or urgent clinical concerns require an appropriate healthcare or emergency pathway rather than the complaint queue.

5. Refund Principles

Refund eligibility depends on the recorded payment, service outcome, cancellation timing, and the applicable service rule. Typical review cases include duplicate payment, a service cancelled within an eligible policy window, or a documented service failure for which the approved billing policy allows adjustment.

Prices and payment status come from the structured billing or booking record. This document describes the process; it is not an authoritative price list.

6. Non-Automatic Refund Situations

A refund should not be promised automatically when the customer missed a visit, requested a late cancellation, changed their mind after a completed service, or when a test requires recollection for a reason not covered by the refund policy.

When the rule is unclear, create an escalation rather than inventing an exception.

7. Escalation Levels

Level 1: Customer service resolves straightforward process questions using trusted data and policies. Level 2: Operations or laboratory support reviews service-delivery, specimen, or workflow issues. Level 3: Management, billing, privacy, or another designated owner handles disputes or higher-risk incidents.

The case record should show what was checked, what is still unknown, and which team owns the next action.

8. Response and Follow-Up

For issues that cannot be solved in the first interaction, provide a realistic follow-up expectation based on the responsible team's process. Do not invent an SLA when one is not defined.

When an issue is resolved, record the final action, reference any refund or replacement booking actually created, and avoid claiming completion before the system reflects the change.

9. Human Handoff

The AI agent should hand off when required information is missing after bounded clarification, when the customer disputes system records, when a policy exception is requested, or when the issue involves privacy, safety, or another high-risk concern.

The handoff summary should include the user's request, confirmed facts, relevant IDs, what has already been attempted, and the unresolved question.

10. Clinical Safety Boundary

MediLab customer service does not diagnose, interpret laboratory values, recommend treatment or medicines, choose medically necessary tests based on symptoms, or provide emergency triage.

When a customer asks for clinical advice, the response should explain the boundary and direct the customer to a qualified healthcare professional.

Document Control Notes

- This is a fictional MediLab assessment document designed to resemble real diagnostic-laboratory customer-service documentation.
- If a live structured record conflicts with this document, the structured record controls for price, availability, booking, status, and test-specific catalog values.
- If a future policy changes, update the KnowledgeDocument version and reindex the RAG corpus so stale chunks are removed.
- Customer service must not use this document to diagnose, clinically interpret results, prescribe medication, or recommend medically necessary tests from symptoms.